

August 12, 2026

UnitedHealthcare
Provider Appeals
P.O. Box 30432
Salt Lake City, UT 84130

RE: Formal Appeal of Claim Denial — Yi Weissnat (MRN MUSC5718072), Claim MUSC-20260616-F39B-1

Patient	Yi Weissnat (DOB 2002-12-22)
MRN	MUSC5718072
Member ID / Group	UHC206518920 / GRP-81932
Claim number	MUSC-20260616-F39B-1
Date of service	2026-06-16
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$1,080.46
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2027-07-12

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health submits this first-level appeal on behalf of patient Yi Weissnat (MRN: MUSC9727193, Member ID: UHC206518920, Group: GRP-81932) regarding the denial of Claim MUSC-20260616-F39B-1 for services rendered on June 16, 2026, at MUSC Health Ashley River Tower by rendering provider Priya N. Raghavan, MD (NPI 1669504132). The claim was submitted on June 27, 2026, and denied on July 12, 2026, in the total billed amount of \$1,080.46. UnitedHealthcare denied the claim under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") with RARC N115 ("This decision was based on a Local Coverage Determination (LCD)"), further stating that "documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests full reversal and payment of the claim.

CLINICAL BACKGROUND

Ms. Weissnat is a 23-year-old established patient with a documented history of acute viral pharyngitis, consistent with the primary diagnosis of J06.9 (Acute upper respiratory infection, unspecified) reported on the claim. Her medical record reflects prior episodes of acute viral pharyngitis, prior throat culture, and prior antibiotic therapy, establishing a relevant clinical history for upper respiratory illness. The June 16, 2026, encounter was billed as two units of CPT 99214, an established outpatient office visit of moderate complexity, at Place of Service 22 (On Campus Outpatient Hospital), under revenue code 0510. The level of service reported is consistent with the evaluation and management of an acute respiratory condition in a patient with a known history of similar illness.

BASIS FOR APPEAL

The denial characterizes the services as not medically necessary; however, the evaluation and management of an acute upper respiratory infection in an outpatient setting is a well-established, standard-of-care service. CPT 99214 is appropriately assigned when the medical decision-making or time requirements for a moderate-complexity established-patient visit are met. The diagnosis of J06.9 does not, in itself, preclude medical necessity; rather, medical necessity is determined by the clinical circumstances of the individual encounter, including the patient's presenting symptoms, history, and the complexity of the evaluation performed. Ms. Weissnat's documented history of recurrent acute viral pharyngitis and prior pharmacologic treatment supports the clinical appropriateness of a thorough evaluation at this visit.

Regarding the RARC N115 reference to a Local Coverage Determination, MUSC Health notes that LCDs are issued by Medicare Administrative Contractors and apply to Medicare fee-for-service claims. This claim involves a commercial HMO member covered under a UnitedHealthcare plan with a coverage period of January 1, 2024, through December 31, 2026. Application of an LCD framework to a commercial claim requires explicit contractual or plan-document authority. MUSC Health requests that UnitedHealthcare identify the specific coverage policy or contractual provision upon which this denial is based, as the invocation of an LCD in this context appears to be inapplicable. To the extent that UnitedHealthcare's internal medical necessity criteria were applied, MUSC Health requests that those criteria be disclosed so that a complete clinical response may be provided.

REQUESTED ACTION

MUSC Health respectfully requests that UnitedHealthcare conduct a full clinical review of this appeal, reverse the medical necessity denial, and issue payment of the claim in accordance with the contracted allowed amount. Should UnitedHealthcare require additional clinical documentation, operative notes, or a peer-to-peer discussion with Dr. Raghavan, MUSC Health will promptly facilitate that process. We further request written notification of the appeal determination within the timeframe required under applicable state and federal prompt-pay and appeal-rights obligations. This appeal is submitted electronically via the Claims & Payments portal as directed in the denial correspondence, and MUSC Health reserves all further appeal rights pending the outcome of this reconsideration.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record